

BME80G Final Part II: Writing Assignment & Oral Debate Preparation

Name: Sage Clokey **Topic:** Telehealth Expansion **Side:** FOR permanent expansion of telehealth as a standard mode of care delivery

Part A. Title

Telehealth Should Be Permanently Expanded as a Standard Mode of Care Delivery: Moving Medicine from the Institution to the Individual

Part B. Background (~400 words)

Telehealth — the delivery of clinical services via video, phone, or digital platforms — existed before COVID-19 but was marginal: only approximately 1% of healthcare visits in early 2020 and 14.3% of physicians reporting any use in 2018. The pandemic forced a massive natural experiment. By April 2020, telehealth surged to nearly 50% of all visits, a 766% increase. Medicare telehealth visits jumped from 5 million to over 53 million. Emergency regulatory waivers made this possible, relaxing interstate licensure requirements, HIPAA enforcement, and controlled substance prescribing rules under the Ryan Haight Act (Patel et al., *Telemedicine and e-Health*, 2022).

Now that the emergency is over, the central ethical question is whether telehealth should become a permanent, standard mode of care or revert to its pre-pandemic restrictions. This is contested because it touches on competing values: access and autonomy versus quality and safety; innovation versus regulation; patient convenience versus the integrity of the clinical encounter.

Post-pandemic, 74% of physicians still use telehealth (compared to 14% pre-pandemic), and 80% plan to continue (Patel et al., 2022). Meta-analyses show telehealth is clinically non-inferior to in-person care for depression, anxiety, primary care management, and chronic disease monitoring (Fernandez et al., *Journal of Psychiatric Research*, 2021; Ramaswamy et al., *Journal of General Internal Medicine*, 2024). However, disparities in broadband access, digital literacy, and device ownership persist — particularly among elderly, rural, low-income, and non-English-speaking populations (Ortega et al., *JAMA Network Open*, 2022).

The populations most affected include rural patients (94% would need to travel 70+ miles for specialist care without telehealth), elderly and disabled individuals with mobility barriers, mental health patients facing provider shortages, and low-

income workers who cannot take time off for in-person visits. The values genuinely in conflict are:

- **Autonomy and access** versus **paternalistic quality standards** — Should patients choose how they receive care, or should regulators define what counts as a "real" visit?
- **Innovation and efficiency** versus **safety and oversight** — Telehealth visits are five times cheaper, but concerns remain about misdiagnosis, data privacy, and continuity of care.
- **Equity** versus **the digital divide** — Telehealth can reach underserved populations, but only if broadband and device access are addressed.

After reading this background, both sides of the debate should agree these facts are accurate and that the ethical tension is real.

Part C. Prepare Your Argument

Thesis statement: Telehealth should be permanently expanded as a legally protected, standard mode of healthcare delivery because it returns medical decision-making to the patient-provider relationship, improves outcomes for underserved populations, reduces systemic costs, and moves healthcare from a centralized institutional model toward a distributed, human-centric architecture — the same architecture that living systems use to coordinate care, and the same architecture that fraternal societies already proved works.

Premise 1: Telehealth restores patient autonomy by decentralizing the point of care — solving the knowledge problem Hayek identified in all centralized systems.

The hospital-centric model concentrates all of healthcare at a single node. Every patient must travel to that node, wait at that node, and submit to that node's schedule. This is the architecture of central planning — all information and decision-making flow through one point. Hayek showed in 1945 why this fails: "The knowledge of the circumstances of which we must make use never exists in concentrated or integrated form, but solely as the dispersed bits of incomplete and frequently contradictory knowledge which all the separate individuals possess." No single institution can hold the distributed knowledge of millions of patients — their schedules, their environments, their daily realities.

Telehealth distributes care to where the knowledge actually lives — with the patient. The provider sees the patient in their home environment. The encounter happens on the patient's terms. 94% of rural patients would need to travel over 70 miles for specialist care without telehealth, and 84% would miss a full day of work (Darrat et al., *Telemedicine and e-Health*, 2022). The VA found veterans

using tele-emergency services were nearly half as likely to visit an ED in person. Ascension Health found approximately 70% of virtual care patients would have otherwise gone to urgent care or the ED — meaning the centralized model was creating unnecessary institutional contact, not because patients needed it, but because no alternative existed.

This mirrors how living systems actually coordinate. In a living organism, the immune system does not make the pathogen travel to a central processing facility. The immune response goes to the site of infection. T-cells circulate and travel to where they are needed. Dendritic cells pick up antigens locally and carry information back. The entire defense system is distributed — sensing happens everywhere, response travels to the point of need. The hospital-centric model is like asking every cell with a problem to travel to the spleen. It is absurd at the biological level, and it is equally absurd at the healthcare level.

Premise 2: Telehealth achieves clinical non-inferiority while dramatically reducing costs — you get both quality and access.

This is not a tradeoff. A meta-analysis of 12 studies found no significant difference in effectiveness between telehealth and in-person treatment for depression; 9 studies confirmed the same for anxiety (Coughtrey et al., *JMIR Mental Health*, 2023). Patient satisfaction across 247,087 surveys was non-inferior to in-person care (Ramaswamy et al., *JGIM*, 2024). Telehealth visits are five times less costly; patients are billed \$400 less on average (Ashwood et al., *Health Affairs*, 2020). CMS estimates \$60 million saved in Medicare travel costs alone, projected to \$170 million by 2029. For geriatric patients, cost reductions ranged from \$223 to \$3,846 per event, up to 94% savings in low-income settings.

When something works as well, costs a fraction as much, and the patient prefers it, restricting it requires very strong justification. The burden of proof falls on those who would limit access, not those who would expand it.

Premise 3: Nothing about complex care actually requires a hospital building — the "you can't do a physical exam through a screen" objection misses the real question: who travels?

The question was never telehealth versus in-person. The question is: who travels? In the old model, the patient always travels — even when they are sick, elderly, disabled, or rural. In the distributed model, telehealth handles the approximately 90% of encounters that do not require physical presence, and when a physical exam or procedure is needed, the doctor comes to the patient. The hospital becomes the last resort, not the first stop.

This already works. DispatchHealth covers approximately 80% of what an ED can treat in the patient's home — 97% patient satisfaction, saving payers \$1,000+ per avoided ER visit and \$7,000 per avoided hospitalization. CMS's Acute Hospital

Care at Home initiative (398 hospitals across 39 states) treats pneumonia, heart failure, and COPD exacerbations at home with lower mortality than brick-and-mortar hospitals, lower infection rates, and lower readmissions. 95% of patients rated it 9 or 10 out of 10. The VA's Home-Based Primary Care program reduced hospital days by 60%, readmissions by 21%, and nursing home use by 89%.

Even surgery no longer requires a building. In June 2025, an AdventHealth surgical team performed a robotic prostatectomy on a patient 7,000 miles away in Angola — the longest-distance telesurgery ever completed. Mobile surgical units carry full operating rooms on wheels. Tele-ICU allows remote intensivists to monitor critically ill patients in real time, achieving a 68% reduction in ER visits for high-acuity patients. Handheld ultrasound fits in a pocket; portable CT scanners are FDA-cleared. The hospital is not a medical necessity — it is an architectural habit inherited from an era when equipment could not move and information could not travel. Both constraints are gone.

Premise 4: Decentralized healthcare already worked once — fraternal societies proved it — and it was deliberately destroyed by the same institutional forces now resisting telehealth.

Before hospitals became the center of American medicine, healthcare was already decentralized, affordable, and voluntary. In the late 1800s and early 1900s, fraternal societies (mutual aid societies) were the primary source of healthcare for working-class Americans. By 1920, over one in four adult Americans were members — cutting across race, class, and gender. A lodge would contract with a doctor on retainer. Members paid \$1–2 per year (about one day's wages) and received a doctor's care for the entire year, including basic surgery and house calls. No insurance companies, no hospital bureaucracies, no billing departments. The patients controlled the arrangement — they hired the doctor, set the terms, and could replace him if service was poor (Beito, *From Mutual Aid to the Welfare State*, 2000; Long, "How Government Solved the Health Care Crisis").

This was not just for white men. The Free African Society of Philadelphia (1787), founded by Richard Allen and Absalom Jones, provided sick benefits and community support. The Knights and Daughters of Tabor opened Taborian Hospital in Mississippi in 1942 because Black Americans were routinely denied admission to white hospitals — membership nearly doubled to 47,000 in three years. These communities did not wait for the government to solve their problem. They solved it themselves through voluntary association.

The AMA destroyed this system. They called it the "lodge practice evil," sanctioned any doctor who accepted lodge contracts, pressured hospitals to deny privileges to lodge doctors, and gained control of medical school accreditation — reducing the number of medical schools from 166 to 81 between 1904 and 1918, restricting doctor supply and driving up prices. The lodge doctor who made house

calls for \$2 a year was replaced by the hospital that charges \$2,000 for an ER visit. That was not progress. That was cartelization.

Telehealth is the modern lodge practice. It does the same thing fraternal societies did: the doctor comes to the patient, the cost drops dramatically, access expands to underserved communities, and the patient controls the encounter. The historical parallel is exact — and so is the institutional resistance. The question for this debate is not whether decentralized healthcare can work. History already answered that. The question is whether we will let it be destroyed again.

The deeper structural insight is about incentives. The modern insurance company is structurally separated from health itself. The insurer profits by denying claims — UnitedHealth's nH Predict algorithm (discussed in class) doubled their denial rate, yet 90% of those denials were reversed on appeal, but only 0.2% of patients ever appealed. The hospital and pharmaceutical system profits when people stay sick — chronic disease management is recurring revenue. A cured patient is a lost customer. The patient is trapped between two entities that both profit from illness. The fraternal society unified what insurance separated: the same body that pooled money also hired the doctor and also lived with the consequences. The only way to win was to keep people healthy. Telehealth restores this alignment — a direct, voluntary exchange between patient and provider, with an honest price signal and no middleman corrupting it.

The biological version: in a living cell, a signaling molecule connects directly to its receptor. There is no middleman molecule sitting between ligand and receptor collecting a fee. When you insert something that corrupts that signal — an oncogene that constitutively activates a receptor regardless of actual demand — you get cancer. Growth without information. Activity without purpose. The insurance company is the oncogene of healthcare: a false signal generator that disconnects the system from the information it needs to function.

Normative ethical theory: Utilitarianism.

Under utilitarianism, the ethically correct action maximizes well-being for the greatest number. Telehealth expansion satisfies this across every measurable dimension: more people access care (access), outcomes are equivalent or better (quality), costs are lower for patients and systems (efficiency), satisfaction is high (preference), and ED burden decreases (systemic benefit). Restricting telehealth to protect a traditional care model that serves fewer people at higher cost produces less total well-being. The utilitarian calculus is clear: permanent expansion maximizes aggregate health outcomes while minimizing aggregate costs and barriers.

Secondary framework — Contractarianism: Behind a Rawlsian veil of ignorance, not knowing whether you would be rural or urban, wealthy or poor, mobile or disabled, you would choose a healthcare system that includes

telehealth as a standard option. The "least advantaged" members of society — rural, elderly, disabled, low-income — benefit most from telehealth. Restricting it harms precisely those a just society would prioritize.

Part D. Anticipated Q&A Responses

Q1: "Doesn't telehealth compromise diagnostic accuracy? You can't do a physical exam through a screen."

Response 1: This is the strongest objection, and it is partially valid — certain conditions require hands-on examination. But the question assumes telehealth replaces all in-person care, when it actually supplements it. Telehealth is most effective for follow-ups, chronic disease management, mental health, medication adjustments, and triage — encounters where the physical exam adds minimal diagnostic value. Meta-analyses confirm no significant difference in outcomes for depression, anxiety, hypertension, and diabetes monitoring. More importantly, the real answer is not "telehealth instead of physical exams" — it is "so the doctor comes to you." DispatchHealth covers 80% of ED-treatable conditions in the home. CMS's Hospital at Home program achieves lower mortality than brick-and-mortar hospitals. Telesurgery works at 7,000 miles. The question was never telehealth versus in-person. The question is who travels — and it should not be the sick person.

Q2: "What about the digital divide? Doesn't telehealth just help people who already have good internet and devices?"

Response 2: This is a real concern — and it is an argument for investing in broadband infrastructure, not for restricting telehealth. Disparities in digital access are a problem to solve, not a reason to deny telehealth to the millions who can already benefit. The FCC's broadband programs, Lifeline subsidies, and community health center hotspots are all mechanisms to close this gap. Meanwhile, telehealth has already demonstrated effectiveness for racial and ethnic minorities when delivered in the patient's preferred language (Connolly et al., *JAMIA*, 2022). The alternative — keeping healthcare locked inside institutions that many underserved patients cannot physically reach — does not solve the access problem either. It just keeps the barrier physical instead of digital. At least the digital barrier is fixable with infrastructure investment. The physical barrier of geography is permanent. The ethical response is to expand telehealth and close the digital divide simultaneously, not to hold one hostage to the other.

Q3: "How do you ensure data privacy and HIPAA compliance when care happens on personal devices and home networks?"

Response 3: HIPAA already applies to telehealth — providers must use encrypted, HIPAA-compliant platforms such as Zoom for Healthcare, Doxy.me, and Epic's MyChart Video. The privacy concern is valid but not unique to telehealth: in-person medical records are digitized and stored in the same electronic health record systems. The data risk is in the storage and transmission infrastructure, not in the mode of the visit. In fact, telehealth can enhance privacy in some cases — patients discussing sensitive topics like mental health, sexual health, or substance use may feel more comfortable in their own homes than in a waiting room. The regulatory framework exists. The question is enforcement and investment in secure platforms, not whether telehealth should be allowed. Restricting telehealth over privacy concerns while accepting that in-person visits use the exact same digital records infrastructure is an inconsistent standard.

Q4: "Doesn't telehealth weaken the patient-provider relationship? Medicine is built on human connection and trust."

Response 4: This assumes the relationship depends on physical co-presence, but the evidence contradicts this. Patient satisfaction with telehealth is non-inferior to in-person care across 247,000+ surveys (Ramaswamy et al., 2024). For mental health, 67% of patients no longer had clinically significant anxiety and 62% no longer had clinically significant depression after approximately five telehealth appointments (Fernandez et al., 2021). The therapeutic relationship formed and produced measurable results. Furthermore, telehealth actually strengthens continuity of care — patients who would otherwise cancel due to transportation, childcare, or work conflicts can maintain regular contact with their provider. A relationship that continues is stronger than one interrupted by barriers. Fraternal society lodge doctors understood this — the doctor who made house calls and knew his patients in their own homes had a deeper relationship than the specialist you see once in a sterile office. Telehealth restores that continuity.

Part E. Opposition Analysis (~300 words)

Opposing Argument 1: "Telehealth creates a two-tier system — high-quality in-person care for the wealthy, inferior virtual care for everyone else."

This is a serious equity concern. If telehealth becomes a cost-cutting substitute rather than a genuine alternative, it could stratify care quality.

Rebuttal: The two-tier system already exists — it is called geography. Rural patients, low-income urban patients, and those without transportation already receive less care, less frequently, with longer delays. Telehealth does not create stratification; the hospital-centric model does. The data shows telehealth outcomes are clinically equivalent for the conditions where it is appropriate. Expanding telehealth flattens the hierarchy — it does not create one. And

historically, fraternal societies proved that decentralized, affordable care served everyone the mainstream system excluded, including African Americans routinely denied hospital admission. The two-tier system was created by centralization, not by distribution.

Opposing Argument 2: "Telehealth cannot replace the physical examination, and normalizing virtual visits will lead to missed diagnoses and worse outcomes."

Some conditions genuinely require physical examination — abdominal emergencies, cardiac auscultation, and certain dermatological assessments.

Rebuttal: No serious advocate argues telehealth should replace all in-person care. The argument is for telehealth as a standard option within a fully distributed model — telehealth for the 90% of encounters that do not require hands-on assessment, mobile providers and house calls for the rest. CMS's Hospital at Home program already treats acute illness at home with lower mortality than hospitals. DispatchHealth covers 80% of ED-treatable conditions in the home. The missed-diagnosis concern is real for a narrow set of presentations and is addressed by provider discretion and mobile medicine, not blanket restriction.

Opposing Argument 3: "Rapid telehealth expansion without adequate regulation risks fraud, overprescription, and exploitation by telehealth-only companies prioritizing volume over quality."

There are documented cases of high-volume, low-quality telehealth operations that churn through patients for profit.

Rebuttal: This is a regulation problem, not a telehealth problem. Fraud and overprescription exist in brick-and-mortar medicine too — pill mills, unnecessary surgeries, upcoding. The DEA's proposed Special Registration for Telemedicine and the Interstate Medical Licensure Compact's 42-state framework show regulatory infrastructure is catching up. The answer to bad actors is better oversight, not restricting a technology that 74% of physicians use. Fraternal societies solved this by giving the patient the power to fire the doctor. The modern system took that power away. Telehealth returns it.

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